INDICATION: A 6-year-old child presenting with severe sleep-disordered breathing, observed nocturnal apneas, heavy snoring, and recurrent bouts of acute streptococcal pharyngotonsillitis.

PROCEDURE: Coblation-assisted tonsillectomy and adenoidectomy.

SURGICAL NARRATIVE: The patient was administered general endotracheal anesthesia using an oral RAE endotracheal tube secured in the midline. The patient was placed in the Rose position with a shoulder roll, and a Crowe-Davis mouth gag was carefully inserted and suspended to visualize the oropharynx.

The uvula and soft palate were retracted anteriorly using two 8-French red rubber catheters passed through the nares. Mirror examination of the nasopharynx confirmed massive grade 4 adenoid hypertrophy completely obstructing the choanae. Using an ArthroCare Coblation wand with continuous saline irrigation (settings: 7 ablation, 3 coagulation), the adenoid pad was completely ablated down to the prevertebral fascia while maintaining meticulous clearance from the torus tubarius and Eustachian tube orifices. Excellent hemostasis was achieved.

Attention was redirected to the palatine tonsils, which were hypertrophic grade 4 (kissing tonsils). The superior pole of the right tonsil was grasped with an Allis clamp and medialized. Using the coblation wand, an incision was made in the anterior tonsillar pillar, identifying the true capsular plane. The tonsil was dissected free from the superior pole to the inferior pole along the capsule, simultaneously coagulating supplying perforators from the tonsillar branch of the facial artery. The left tonsil was resected in an identical fashion. The tonsillar fossae were inspected; complete hemostasis was confirmed with bipolar electrocautery. The oropharynx was copiously irrigated and stomach contents evacuated via an orogastric tube. The mouth gag was released, and the patient extubated awake and stable.